



Mental Health Division

Mental Health Unit (MHU)

Royal Perth Hospital

Model of Care

Traditional Owners Acknowledgement

The East Metropolitan Health Service recognises the Whadjuk People of the Noongar Nation as the traditional custodians of the land on which we learn and work today.

We acknowledge the Whadjuk people have a continuing spiritual and cultural connection to this land and we pay respects to Elders past, present and future.

MHU Model of Care

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Method of Development

The development of this document involved consultation with mental health clinicians, subject matter experts, representatives from key service organisations and the Royal Perth Bentley Group (RPBG) MHU Clinical Working Group (CWG) under the leadership of Dr Alex Jaworska, Mental Health Division Medical Co-Director. Membership of the CWG is documented in the group's Terms of Reference and included multidisciplinary clinical leads.

Strategic Alignment

- Australian Commission on Safety and Quality in Healthcare (ACSQHC; 2017). National Safety and Quality Health Service Standards. Sydney: ACSQHC
- Commonwealth of Australia (2010). National Standards for Mental Health Services 2010. Canberra: Commonwealth of Australia
- Commonwealth Government of Australia (2016). The Fifth National Mental Health Plan [draft for consultation]. Canberra: Commonwealth Government of Australia
- Council of Australia Governments (2012). The Roadmap for National Mental Health Reform 2012-2022. Canberra: Council of Australian Governments
- Department of Health (2010). Clinical Services Framework (CSF) 2010-2020 (and Update, September 2012). Perth: WA Department of Health
- Mental Health Commission (2015). Western Australian Mental Health, Alcohol and Other Drugs Services Plan 2015 -2025. Perth: Mental Health Commission
- Mental Health Act (MHA), WA 2014
- OCP Standards for Authorisation of Hospitals under the Mental Health Act 2014 (2019)
- Charter of Mental Health Care Principles (MHA 2014)





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Definitions

Involuntary Consumers

Involuntary consumers are consumers with a mental illness who are placed by a psychiatrist on an involuntary order under the provisions outlined in the Mental Health Act, 2014.

1 Introduction Overview and Purpose

The MHU Model of Care (MOC) describes the proposed philosophy and operational approach to integrated, early stage, recovery oriented, consumer driven care. The purpose of this model is to guide and support the development of evidence-based practices which aligns with the overall strategic direction of mental health services at RPBG.

To ensure its relevance, the MHU MOC has been developed through collaboration with mental health clinicians, subject matter experts, and representatives from key service organisations. Further, responsible executives have been involved to ensure strategic alignment and research has been conducted to capture local and global best practices within relevant fields. The practices shaped by the collaborative process aims to help:

- support the delivery of consumer-centred and trauma informed care practices for consumers with complex mental illnesses and challenging behaviours, by informing staff training and support, and strengthen relationship with important services such as the Alcohol and Other Drug (AOD)
- support the improvement of consumer recovery journeys, by further strengthening service integration between step down facilities, community services, and nongovernment organisations (NGOs)
- improve consumer flow efficiency and quality, by developing and implementing new informed policies and standard operating procedures
- improving staff, consumer, and visitor safety, by upskilling staff and guiding the development of a fit-for-purpose involuntary ward unit
- further support the delivery of care practices for identified unique consumer groups.

Key strategic planning documents will be based around this MOC throughout the commissioning of the RPH MHU. The MHU MOC is an iterative, best practise document that will be regularly reviewed and refined in the lead up to the opening of the MHU. Information may be updated as new information and circumstances emerge. The plan does not specify staffing, resources or budget parameters but indicates what is proposed and will be required for the current model of care described. Workforce and other resourcing budget priorities may impact the service delivery model proposed.

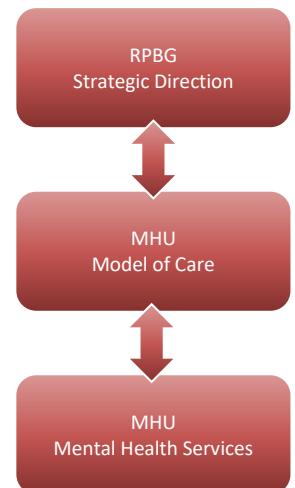


Figure 1 - MHU Model of Care relationship to Strategy and operations



1.1 Mental Health Services at a glance

17%

of patients were readmitted within 28 days of discharge during 2018-19. National average was 14.6% in same time period.

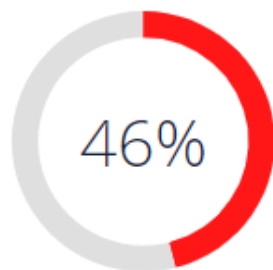
WA's readmission rates have improved from 18.7% in 2017.

2.5X

higher suicide rate amongst Aboriginal and Torres Strait people in WA, compared to non-indigenous people (2015-2019).

Comparison rate is 55% higher in WA than national average.

Expenditure



increase in recurrent expenditures on specialised mental health services between 2009-2019.

National average was 34%

Patient days

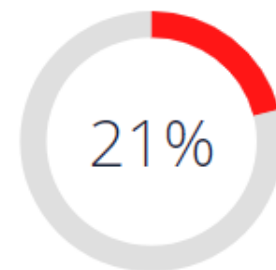
In acute inpatient units has increased by..

12%

..between 2015-2019.

National increase in same time period was only 3%.

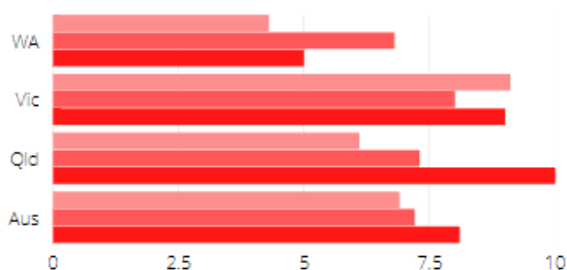
Suicide rates



higher than national average counted between 2015-2019.

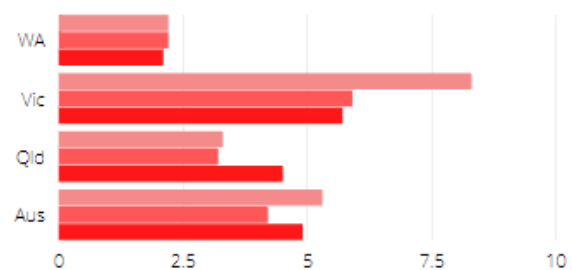
Seclusion and Restraint

No. Seclusion Events



WA reports the least seclusion and restrain events of all Australian states, per 1000 bed days(2017-2020).

Avg. duration of events (hr)



WA showcased the lowest seclusion duration of all Australian states (2017-2020).



1.2 A system under pressure

In recent years, the WA mental health system has been subject to a growing pressure from consumers with severe and complex mental illnesses, with studies showing that 10 % of consumers use up 90% of provided inpatient care¹. It is perhaps not so that Western Australia is experiencing a surge of mentally ill individuals, but instead that the mentally ill are not being properly treated, which results in high readmission rates and escalating needs². The reasons behind this are complicated however, with WA facing unique challenges from its isolated and widely spread out healthcare system and related skills shortages.

In 2019, 17% of individuals discharged from an acute psychiatric unit in WA were readmitted within 28 days. Although the number has improved from 18,7% in 2017, it is still well above the national average of 14.6%³. Further, between 2015-2019 the number of consumer days in acute inpatient units increased by 12% (compared to 3% nationally) and WA's Emergency Departments experience the longest waiting times for mental health presentations^{3, 4}.

A reason for the current situation relates to an historic approach of providing 'episodic' treatment for consumers with acute mental health symptoms, with the primary focus on symptom stabilisation and late stage rehabilitation¹. The problem with this approach is that it doesn't consider unique consumers' needs and neglect preventive efforts. Consequently, people suffering from mental health illnesses aren't properly treated which results in ever escalating symptoms and needs for frequent readmissions.



Figure 2: Traditional path to rehabilitation and recovery services¹

1.3 Towards a recovery-focused treatment model

To address the current situation, a transition towards recovery-focused treatment model, which acknowledge unique consumer needs and supports long term recovery, is proposed¹. The model aims to enable preventive efforts by identifying consumer illnesses in an earlier stage and provides rehabilitation services in earlier stage for consumers with escalating needs.

The value of a recovery-focused service approach cannot be understated. In a study by Lavell, Ljaz and Killaspy (2011) it was concluded that people who receive treatment and care from rehabilitation services, offering integrated treatment, are eight times more likely to achieve/sustain community living when compared to those who are supported by generic community mental health teams. Further, two thirds of people being treated by rehabilitation services achieve successful community living within 18 months of admission, two-thirds sustain this over five years, and around 10% achieve independent living within this period^{5,6}. These efforts could significantly address the growing need for bed days in WA who, in between 2013 to 2017, had 126 people spending 365 or more consecutive days in acute inpatient units at an estimated cost of \$115 million¹.

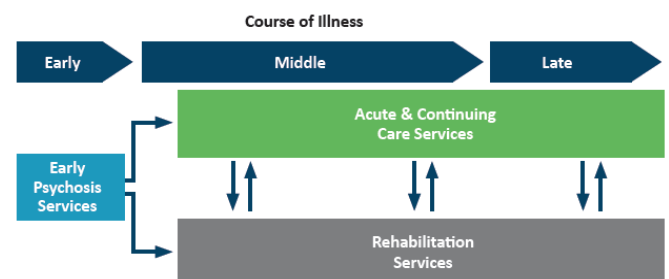


Figure 3: Continuous access to rehabilitation and recovery services¹



1.4 Introducing a new Mental Health Unit at RPH

The RPBG provides acute in-consumer psychiatric care for adult public consumers. As of February 2021, RPH's acute mental health services includes:

- A Consultation Liaison service in-reaching to general and specialist wards;
- An eight-treatment space Mental Health Emergency Centre (MHEC); and
- 20 bed, non-authorized mental health unit, Ward 2K.

Planned to be commissioned by mid-2022 is a new authorised MHU. The MHU is part of the Labour Government's mental health commitment at RPBG, aimed at addressing existing problems such as high readmission rates, lack of service integration, and an 'episodic' approach to mental health care that isn't consumer-centred, and rehabilitation focused.

The MHU will offer a flexible facility and environment configuration, based on the individualised needs of up to 12 consumers requiring a therapeutic environment in which to receive acute, ward-based treatment and care. Considerations include involuntary consumers with needs related to sexual safety, exceptional vulnerability, co-morbid medical, physical safety or observational needs of each consumer.

For RPH, the MHU will enable a smoother consumer journey as consumers placed under the MHA 2014 can now be treated on-site. This will not only help provide consumers with a wider service offering on-site, but also enable a safer care environment for consumers, staff, and visitors.

2 Service Overview

2.1.1 Criteria for Admission

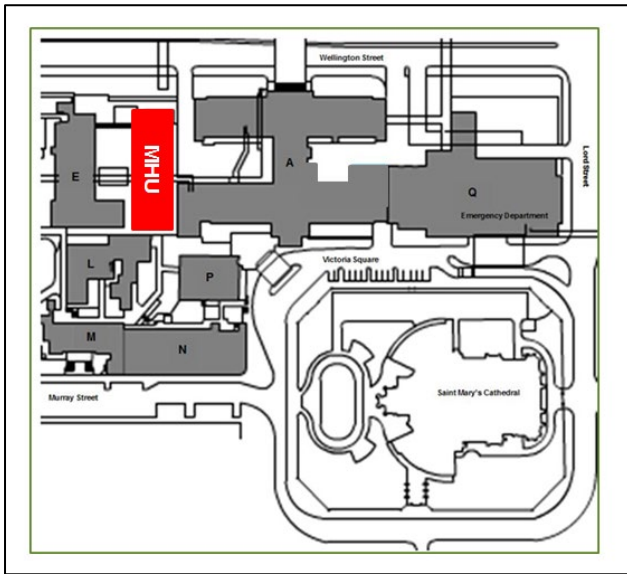
MHU is suitable for involuntary consumers aged 18 – 65 years experiencing acute mental health illness and reside in the RPBG catchment area.

For further information relating to criteria for attendance, refer to the **RPBG MHU Referral, Admission, Discharge and Transfer Policy**.





2.1.2 Location



The MHU is located in B Block, Level 2, RPH Wellington Street Campus. This area was formerly known as 2L.

2.1.3 Hours of Operation

The MHU operates as an acute 24-hour inpatient service, 7 days a week, for involuntary consumers with acute mental health conditions.

2.2 Values

The MHU supports RPH's values of:



The MHU is also committed to serve its consumers, staff and the organisation through abiding by the SERVIO philosophy.



2.3 Mission

Deliver What Matters Most

The MHU will offer a person centred, recovery focused service that considers the consumers presentation, background and goals. The service will have a platform of kindness, integrity and respect.

Consistent High Quality Care

The MHU will deliver best practice care in a therapeutic environment designed to support recovery. The MHU team includes clinical and non-clinical staff focused on an optimal consumer experience.

Culture of Continuous Improvement

The MHU will evaluate and reflect on its performance, act on opportunities and support collaborative problem solving / co-design. Accountability and excellence are key values of the unit.

No Consumer Harm

The MHU prioritises the safety of consumers, visitors and staff. This stems from the units design, a focus on safe practices (e.g. least restrictive practice) and a shared expectation of respect on all who attend the unit.

3 Service Principles

3.1.1 Recovery-oriented service approach

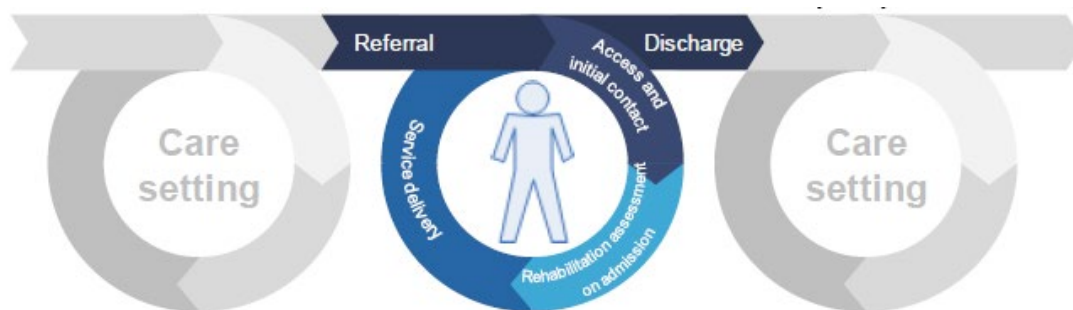


Figure 4 Illustration of consumer rehabilitation journey flow

To address escalating mental health consumer needs and high readmission rates, an 'ongoing' approach to rehabilitation and recovery has been proposed^{1,2}. The approach acknowledges the unique needs of mental health consumers, the fact that some conditions need life-long support, and that interventions provided earlier in the mental health illness cycle often lead to more manageable conditions in later stages¹.

Central to enable the approach are the availability of an integrated rehabilitation and recovery system network [1]. This network will enable rehabilitation journeys that are inclusive of consumer needs and provides support over the long term.

The MHU will support the above approach within RPBG by expanding service offerings for consumers with the most urgent and complex care needs. The MHU will aim to make consumers more susceptible to long term rehabilitation by treating acute symptoms, stabilise conditions, and provide integrated discharge processes.



3.1.2 Recognising physical health needs

It is acknowledged that people with mental illnesses suffer from long-term health conditions in a larger extent than people without mental illnesses³. It is recommended that mental health services consider physical health aspects in their assessment practices, such as medication side effects, lifestyle (exercise, diet, smoking, dental, cholesterol), Physical disorders, Substance use, and Psychosocial issues (family relationships, community involvement, socio-economic status, culture/religion).⁷

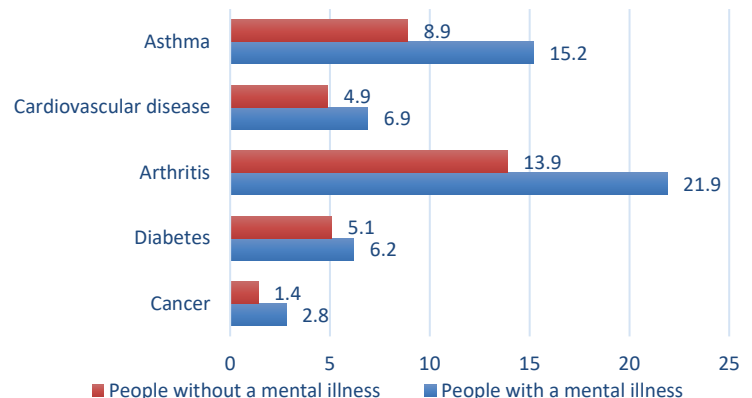


Figure 5 Proportions of adults with long-term health conditions in WA, 2017-18 (Australian Government, 2021)

In this regard, the MHU aims to:

- Consider current medication and prescription, likely side-effects and management
- Offer outdoor exercise options and activities
- Provide healthy food options
- Offer a smoke-free environment by utilising smoking cessation programs and in reach from population health programs that deliver these services
- Engage with consumers to understand socio-economic status, family relationships, and living situation, which will feed into more successful discharge and prevention of 28 day readmission processes.

3.1.3 Supporting integrated treatment for consumers with co-morbid substance use

Substance co-morbidity is common amongst individuals with severe mental illness, often complicating their treatment and leading to poorer outcomes¹. Evidence has proven that integrated treatment of co-occurring mental health and substance use disorders provides better rehabilitation results than separate treatment, whether offered either in parallel or in sequence⁸. Consequently, considerations should be made regarding how mental health rehabilitation and recovery services can enable an integrated treatment approach for people with severe, enduring mental illness and co-occurring substance use¹.

The MHU recognises that many of the consumers presenting to the unit will suffer from substance co-morbidity and seeks to support an integrated treatment approach by:

- Providing all consumers with active co-morbid substance use specialist AOD consultation as part of their management plan
- Promoting engagement with external service providers to strengthen internal offering
- Providing withdrawal management support to stabilise consumer conditions and improve susceptibility to rehabilitation practices
- Supporting the clinical workforce with upskilling in AOD treatment, to increase awareness and quality of rehabilitation and recovery practices.





3.1.4 Culturally Sensitive Care

The MHU will provide services in a culturally sensitive manner that considers the needs of consumers that identify with sexual/gender/bodily preferences (i.e. Lesbian, Gay, Bisexual, Transgender, Intersex, [LGBTI+]), Aboriginal consumers/ consumers with a disability or from a culturally and linguistically diverse background (CaLD).

Special consideration will be given to ATSI consumers based on their significantly high suicide rates in WA, i.e. Indigenous people were 2.5 times higher than the rates amongst non-indigenous, which is 55% higher than the difference seen on a national level between 2015-2019¹.

The MHU supports

- Rainbow accreditation and further guidance can be found in the refer to **LGBTI+ Inclusivity Policy**
- Where required, staff to consult with the Aboriginal Liaison Officer or Aboriginal and Torres Strait Islander (ATSI) clinician/access traditional healers/elders as per the MHA 2014, if appropriate, to assist with culturally appropriate communication techniques, and be aware of concerns about confidentiality within a cultural group
- Engaging a support person or an accredited interpreter with CaLD consumers who may not speak English/ English is a second language/ Aboriginal consumers
- Consulting with relevant agencies to support consumers specific needs, e.g. Aboriginal Medical Service health professionals, Multicultural Youth Advocacy Network of WA, local GPs and other health services.

Consumers of No Fixed Abode (NFA)

Consumers who are admitted to the MHU may be classified as having no fixed abode (NFA). This could include interstate residents, overseas tourists or young persons who are homeless.

For more information on discharge and treatment processes for NFA consumers, refer to **MHU Referral, Admission, Discharge and Transfer Policy**.

3.1.5 Endorsing a Least Restrictive Practices approach

The MHU is committed to creating and maintaining a safe ward environment. Clear guidelines for appropriate behaviour will be provided to all consumers who are admitted to MHU. Strict behavioural management plans will be developed on a case by case basis as required.

The Aggression Prevention and Intervention (API) model and Safe Wards guidelines will be an essential component of MHU's approach to managing aggression on the ward.

Least restrictive practices, for e.g. graded de-escalation approaches, will be offered across all areas of consumer care to direct and encourage appropriate standards of behaviour whilst consumers are admitted to the unit.





4 Accessing the service

For information on procedures relating to referrals and admission to the MHU, refer to the **RPMG MHU Referral, Admission, Discharge and Transfer Policy**.

5 MHU Service Provision

Underpinned by its service principles, the MHU provides:

- The development of comprehensive individualised management plans with a focus on recovery (where clinically appropriate)
- Specialised multidisciplinary interventions designed to address distress and to ameliorate challenging behaviour/s
- Evidence-based, individual interventions that support acute life skills recovery, assisted problem solving that precipitated admission and improve mental health illness symptomology
- Evidence based, pharmacological management including combinations of anti-psychotics and benzodiazepines as appropriate to relieve distress and achieve low levels of sedative rest
- Comprehensive and integrated management of consumers appropriate for the MHU who have significant medical comorbidities
- Education and psycho-education about mental health and management of mental health issues to the consumer, and their family/carers/guardians and support services;
- Education about physical health and management of issues to the consumer, and their family/carers/guardians and support services
- AOD education and management
- The development of strong linkages with relevant community mental health services to facilitate discharge planning and smooth transitions in coordinated care
- 7-day therapeutic program with standalone modules, based on proven concepts in similar ward areas
- Multi-Agency/Family Meetings to share information, manage risks, and plan future recovery strategies
- Co-morbidity expertise within all care pathways, including a dedicated Aboriginal Co-morbidity Worker position to link mental health and social and emotional well-being services with AOD services.

6 Leaving the service

For information on procedures relating to discharge and transfer from the MHU refer to the **RPMG MHU Referral, Admission, Discharge and Transfer Policy**.





7 Enablers for implementing Model of Care

7.1 Workforce

As a key enabler to the fulfillment of MHU objectives stands its associated workforce, as in essence the MHU is a service provider. To support the recovery and stabilisation for consumers with complex needs, access to evidence-informed interventions is essential. At the MHU the delivery of interventions will be supported by a multidisciplinary team where each profession brings its unique knowledge, skills, experience and perspective to bear. This will help enable a consumer centred approach where consumer conditions and co-morbidities are identified and more appropriately managed over time.

Further, and perhaps even more importantly, the MHU will prioritise the staffing of experienced professionals who genuinely wants to work in an involuntary ward environment with consumers presenting with the most urgent and complex care needs, through appropriate job descriptions and selection criteria. Extra emphasis will be placed on personal qualities such as the ability to engage with consumers and external service providers to build working alliances, support therapeutic optimism and manage frequent setbacks, and overall contribute towards developing a positive work culture at the MHU.

It is recognised that consumers presenting to the MHU may demonstrate challenging behaviours which can pose a risk to staff, visitors, or other consumers, if not appropriately managed. Consequently, the MHU will emphasise the recruitment of experienced staff and offer appropriate upskilling support, to ensure a safe clinical environment which upholds a least restrictive practices approach. This is primarily enabled through the deployment of appropriate de-escalation techniques, the utilising of comfort rooms and de-escalation areas, and having assessment processes in place which captures possible risks and develops mitigation techniques.

7.2 Care coordination and linkages

Coordinated, seamless treatment is particularly important for people with severe, enduring mental illness and complex needs whose recovery journey often necessitates access to and transition across a range of clinical settings - community, residential and inpatient [1]. In this regard, due to the specialised tertiary level of care that the MHU provides, close linkages and engagement will be made with multiple external service providers including:

- RPH ED and Mental health units
- RPH Consultation Liaison (CL) service
- RPH AOD specialists
- RPHG acute inpatient units and CMHS
- Specialist Aboriginal Mental Health Services (SAMHS)
- NGO





7.3 Facility design and infrastructure

It is expected that more than 300 consumers will be treated within the MHU per year based on length of stay for similar consumers at like sites. It is expected that the average length of admission will be between 11 – 15 days meaning the environment must be conducive for longer stay admissions. Further, the MHU will aim to support consumer independence with activities of daily living, reduce potential institutionalization and enable a level of comfort associated with longer stays.

In this regard, the MHU will be designed and structured and operated in a manner that minimises risk for consumers, visitors and staff. Research and consultation have been undertaken to ensure facility design lives up to safety standards aligned with *The Chief Psychiatrist's Standards for Authorisation of Hospitals under the Mental Health Act 2014*.

Key considerations included in facility design are:

- Providing flexible options for accommodating needs for consumer safety including management of high-risk consumers, management of sickness and disease, and sexual safety
- Providing staff with possibilities for appropriate oversight and observation of consumer areas;
- Providing necessary security measures to mitigate risks of self-harm, harm towards others, or consumer absconding (e.g. anti-ligature, anti-barricade, secure storage areas, fittings and fixtures having low breaking strain)
- Utilising modern technologies for enhanced rehabilitation and treatment (e.g. smoking cessation, sensory modulation, observation)
- Providing an appropriate balance between welcoming design and safety features
- Providing consumer access pathways which minimises disruption for other consumers and visitors, while also catering to privacy needs and low stimulus
- Providing designated consumer areas for supporting de-escalation, and stabilising treatment (e.g. comfort rooms, quiet rooms, de-escalation rooms, visitor rooms)
- Providing designated consumer areas for rehabilitation-focused activities (e.g. activity rooms, outdoor courtyards).

7.4 Governance and performance review

7.4.1 Governance

The Authorised MHU governance aligns with RPBG Executive, through Mental Health Division's mental health program based at RPH's Wellington Street Campus.

Operational and professional management of the MHU is the responsibility of the area's Consultant Psychiatrist and Nurse Unit Manager (NUM). The MHU leadership team are responsible for the daily operations of the unit, including management of risks, issues and communication, professional development and safety, quality and continuous improvement.





7.4.2 Key Performance Indicators

The MHU Team is responsible for the collection and reporting of KPIs to measure benefits and enable agile decision making to improve service delivery.

Primary clinical Key Performance Indicators (KPI's) are routinely reported to the monthly Mental Health Division Clinical Quality and Safety meetings include (but are not limited to):

- 7-day follow-up
- 28-day readmission
- Case-mix adjusted Length of Stay (LOS)
- Combined Observational Bed-side Risk Assessment (COBRA) metrics i.e. consumer ID, medication safety, clinical handover
- Consumer experience survey
- Clinical Incident Management System (CIMS) incident report
- Governance, Evidence, Knowledge, Outcomes (GEKO) quality improvement activity report
- RPBG mental health audit results i.e. compliance with State-wide Standardised Clinical Documentation (SSCD).

In addition, several specific clinical indicators will be monitored by the MHU team:

- Percentage of consumer referred that are accepted for admission to MHU
- MHU entry pathways and time from referral to acceptance and then admission
- MHU Occupancy
- MHU discharge pathways and Exit Block
- MHU discharge day of week and time of day
- Emergency Department (ED) LOS for mental health admissions.

In addition to the local reporting and monitoring of clinical KPI's, ad hoc reporting of clinical indicators may be required to the Office of the Chief Psychiatrist (OCP), the East Metropolitan Health Service (EMHS), the Mental Health Commission (MHC) and the Australian Council on Healthcare Standards (ACHS).

7.4.3 Consumer Post Satisfaction

A consumer may choose to provide feedback about the MHU service in the following ways:

- EMHS electronic satisfaction survey
- Verbal in-person feedback
- EMHS/RPBG Feedback forms available onsite
- Care Opinion

Further information: <https://rph.health.wa.gov.au/About-us/Compliments-and-Complaints>

7.4.4 Research and Evaluation

Focused research enquiry and knowledge sharing is essential to provide high quality consumer focused mental health services.

Research endeavours will be actively pursued at MHU so that the model of care is modern, contemporary, evidence based, and relates to the needs of the RPBG, MHU consumer and care population.

Emphasis will be given to both research and evaluation initiatives. Opportunities will be provided to staff to investigate, reflect and improve on practice, and to build research skills which will contribute to reform.





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Document Control

Document History

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0.2	20/9/2018	James MacWatt, Project Manager	Update to reflect feedback from Richard Stewart and James Sherriff
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1.3	16/12/2019	Kirsten Claffey, Project Manager	Updated to reflect service and project changes
1.4	22/01/2020	James MacWatt, Project Manager	Document review and update before tabling to MHU Operational Committee
1.5	12/02/2020	James MacWatt, Project Manager	Document update based on feedback by MHU Operational Committee
2.0	20/3/2020	James MacWatt, Project Manager	Document update based on feedback from consumer consultation provided through Mel Birrell
2.1	07/04/2021	Julius A. Pipirs Graduate Officer	Document revised as part of ongoing work in MHU Project
2.2	27/05/2021	Julius A. Pipirs Graduate Officer James MacWatt, Project Manager	Document revised as part of ongoing work in MHU Project inc. feedback from consultation group

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Distribution for Consultation

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1.4	22/01/2020	MHU Operational Committee
2.0	20/03/2020	MHU Operational Committee
2.1	19/04/2021	MHU Operational Committee, MH Division Safety and Quality, RPH Emergency Department, RPH Operations Hub
3.0	TBC	MHU PCG

Approvals

Version	Executive Sponsor	Signature	Date
0.4	Lesley Bennett	Approved	
2.0	Lesley Bennett	Approved	

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